

Hospice Interview Tool

1. Who Are We Talking To?

Fill in the details below for the hospice you meet with.

Hospice legal name	_____
Branch / location	_____
Date	_____
Time	_____
Person we met (name & title)	_____

Ownership (check one)

- ☐ Nonprofit
- ☐ Hospital-owned
- ☐ Large national for-profit
- ☐ Regional for-profit
- ☐ Private-equity-backed
- ☐ Other: _____

Leaders we can call

- Administrator name: _____
Phone: _____
- Clinical manager name: _____
Phone: _____
- Medical director name: _____
Phone: _____

First impression

- ☐ Clear and direct
- ☐ Warm but vague
- ☐ Sales-y / rushed

Notes:

2. Routine Care – What Are They Actually Promising?

At start of care, they say they will give:

RN Visits

- ☐ 1–2 / week
- ☐ 3–4 / week
- ☐ 5+ / week
- ☐ Wouldn't say / unclear

Aide Visits

- ☐ 1–2 / week
- ☐ 3–4 / week
- ☐ 5+ / week
- ☐ Wouldn't say / unclear

Social Worker

- ☐ At least monthly
- ☐ "As needed" only
- ☐ Unclear

Chaplain

- ☐ At least monthly
- ☐ "As needed" only
- ☐ Unclear

After-Hours

Who answers at 2 a.m.?

- ☐ Hospice nurse from this hospice
- ☐ Call center / answering service
- ☐ Not clear

Expected nurse response time they claimed:

- ☐ Within 1 hour
- ☐ 1–3 hours
- ☐ "As soon as possible" / vague

Agency / Contract Staff

- ☐ They rarely use agency nurses
- ☐ They use agency often
- ☐ They dodged the question

Does this feel realistic?

- ☐ Yes
- ☐ No
- ☐ Not sure

3. Crisis Care – CHC, GIP, Respite

Continuous Home Care (CHC)

- ☒ They clearly explained how someone qualifies
- ☒ They gave a real example
- ☒ They said "we almost never do that"
- ☒ They avoided or blurred the question

General Inpatient (GIP)

Place(s) they would send us for GIP:

- ☒ Named a real unit / facility
- ☒ Just said "hospital" / vague

Same-day GIP approval:

- ☒ They named who can approve (role or person)
- ☒ They could not say who approves

Respite

Facility name: _____ Distance:
_____ miles

- ☒ Driving distance feels reasonable
- ☒ Too far / unclear

Weekend and Night Crises

- ☒ They clearly described how a Friday-night crisis is handled
- ☒ Answer was vague ("we'll see," "depends")

Overall crisis confidence:

- ☒ I believe they can staff crisis care
- ☒ I do NOT believe they can staff crisis care

4. Dates, Signatures, and Revocation

Today's Admission Paperwork

Effective date they wrote or want:

This is:

- ☒ Real start-of-care today
- ☒ Future placeholder date
- ☒ Not clear

If We Change Our Mind Before Care Starts, They Said We Should:

- ☒ Call this person:

- ☒ Send something in writing (email / fax / portal)
- ☒ "Just don't worry about it" (**red flag**)

Revocation (Ending Hospice)

- ☒ They clearly said revocation is our written choice, with a date
- ☒ They admitted nurses / managers sometimes suggest revocation
- ☒ They acted like revocation is just what happens if things get expensive

Our rule (check to remind yourself):

- ☒ We will not sign revocation just because care is expensive or someone is impatient.

5. Discharge and Equipment

Did they talk about discharge or "no longer eligible" at all? ☐ Yes ☐ No

If Yes, Did They:

- ☐ Explain clear criteria for "no longer eligible"
- ☐ Stay vague ("we'll know when we see it")

If They Discharge Us, They Said:

Medications:

- ☐ There will be a plan for how we get them
- ☐ Not clear

Oxygen / equipment (bed, suction, wheelchair, etc.):

- ☐ They will not take equipment until replacements are in place
- ☐ They might take equipment before we have replacements
- ☐ Answer was unclear

Cap / Money Pressure

- ☐ They admitted cap / length of stay is real but "clinical decisions come first"
- ☐ They denied any financial pressure ever
- ☐ They dodged or minimized the question

My Feeling After This Section

- ☐ I trust them not to dump us for money
- ☐ I don't trust them on this

6. Place, Room-and-Board, and Money

Where We Expect the Patient to Be Most of the Time

- ☐ Home
- ☐ Assisted living
- ☐ Memory care
- ☐ Nursing facility
- ☐ Other: _____

We Understand

- ☐ Hospice **DOES NOT** pay our rent or facility room-and-board
- ☐ We still pay for caregivers / facility, even with hospice

If Money Runs Out

☐ They described a realistic Medicaid path for our state

☐ They named who can help with paperwork

☐ They were vague or said "we don't really do that"

Long-Term Care Insurance (LTC)

- ☐ We have an LTC policy
- ☐ We don't have / don't know

If yes, they helped us clarify:

- ☐ Daily benefit
- ☐ Elimination period
- ☐ Which settings it covers

7. Trust Score – Right After the Meeting

Circle 1–5 for each category (1 = terrible, 5 = excellent)

Answering hard questions	1	2	3	4	5
Ownership & leadership clarity	1	2	3	4	5
Routine visits realism	1	2	3	4	5
Crisis care credibility	1	2	3	4	5
Honesty about money & cap	1	2	3	4	5
Overall gut feeling	1	2	3	4	5

Biggest Green Flag

Biggest Red Flag

 For Now, We Label This Hospice As:

☒ Strong candidate

☒ Maybe

☒ Probably not

☒ Absolutely not